BY ORDER OF THE COMMANDER
59TH MEDICAL WING

59TH MEDICAL WING INSTRUCTION
44-133

5 FEBRUARY 2021

Medical

PLAN FOR THE PROVISION OF PATIENT CARE

COMPLIANCE WITH THIS PUBLICATION IS MANDATORY

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RELEASABILITY: There are no releasability restrictions on this publication.

OPR: 59 MDW/SGH2

Certified by: 59 MDOG/CC
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Supersedes: 59 MDWI 44-133, 22 May 2018

Pages: 24

This instruction implements Air Force Policy Directive 44-1, *Medical Operations.* This medical wing instruction (MDWI) outlines practices and procedures for providing patient and community health care throughout the 59th Medical Wing (MDW) and incorporates Department of Defense (DoD) patient safety standards. This plan is consistent with the vision, mission, and values of the 59 MDW. This instruction applies to all personnel assigned, attached, or under contract to the 59 MDW with the exception of personnel assigned to the 959th Medical Group (MDG) at Brooke Army Medical Center (BAMC) and the 59th Training Group. Refer recommended changes and questions about this publication to the Office of Primary Responsibility (OPR) using the AF Form 847, *Recommendation for Change of Publication*. Requests for waivers must be submitted to the OPR listed above for consideration and approval. This Instruction requires the collection and or maintenance of information protected by the Privacy Act of 1974 authorized by 10 U.S.C. 55, *Medical and Dental Care*, and E.O. 9397 (SSN). The applicable SORN F044 AF SG D, and Automated Medical/Dental Record System is available at: **http://dpclo.defense.gov/Privacy/SORNs.aspx.** Ensure that all records created as a result of processes prescribed in this publication are maintained IAW AFI 33-322, *Records Management and Information Governance Program*, and disposed of IAW Air Force Records Information Management System (AFRIMS) Records Disposition Schedule (RDS).

SUMMARY OF CHANGES

This publication has been revised. This rewrite of 59 MDWI 44-133 includes a code of conduct that defines acceptable behavior and behaviors that undermine a culture of safety, addresses the

patient safety risk of patient left without being seen or no-show appointments, describes the AMiON resource for provider communication, and updates sexual assault response algorithms.

1. Overview.

1.1. 59 MDW is a collection of academic primary care and subspecialty medical clinics and the largest ambulatory surgical center in the Department of Defense, servicing a large beneficiary and trainee population including active duty military personnel, activated Reserve, Guard, retirees and their dependents, veterans, secretarial designees and beneficiaries of federal interagency agreements (such as members of foreign services or intelligence agencies), and civilian employees ill or injured on the job.

1.2. The 59 MDW comprises 6 groups including the 59th Medical Operations Group (MDOG), 559 MDG, 59th Medical Support Group (MDSG), 59th Dental Group (DG), 59th Training Group and 959 MDG; the 959 MDG serves as the Air Force component to the integrated facility known as BAMC. This instruction addresses provision of care in all facilities excluding those under operational control by the Army at BAMC.

2. 59 MDW Structure and Responsibilities.

2.1. The Board of Directors (BOD) provides strategic guidance and management oversight for meeting the wing's mission.

2.1.1. The BOD is directly responsible for meeting the health care needs of the beneficiary community. This is carried out through goal-focused allocation of resources supported by business case analyses, continuous process and healthcare improvement activities, risk management, credentialing of medical staff, provision of oversight, policy setting and establishment/implementation of standards for patient care and clinical practice guidelines. The BOD determines types of services provided and types of services purchased from or shared with the civilian and military communities based on business case analysis and applicable law.

2.1.2. The BOD will:

2.1.2.1. Ensure patient rights are respected throughout the organization and that business relationships with patients and business associates are conducted in an ethical manner.

2.1.2.2. Identify and consider needs of beneficiaries in developing patient care programs. Methods used to gather patients' inputs include, but are not limited to, customer surveys and direct feedback.

2.1.2.3. Ensure the implementation of an ongoing, collaborative and effective interdisciplinary program to provide, monitor, evaluate and improve patient care outcomes and organizational process improvement.

2.1.2.4. Provide adequate and appropriate resources to manage the health care of the beneficiary population; ensure health care is delivered safely in compliance with The Joint Commission (TJC) National Patient Safety Goals; and support innovations which improve patient care interventions, promote health and embrace population health principles.

2.2. Responsibilities of Wing Staff Functionals.

2.2.1. The 59 MDW Chief of Medical Staff (SGH) along with the 59 MDOG and 559 MDG SGHs (collectively referred to as SGH) leads day-to-day operations and integrates all medical groups to deliver health care. The Wing SGH provides oversight and guidance to the group SGHs. The SGH oversees Credentials and is responsible for ensuring appropriate credentialing processes are in place, and that the 59 MDW Commander or designee awards appropriate clinical privileges, considering each provider's health status, training, experience and demonstrated performance within the capability of the medical facility. Refer to DHA PM 6025.13, volume 4. *Credentialing & Privileging*. The SGH also oversees Patient Safety, Risk Management, Infection Control, clinical performance of the Independent Duty Medical Technicians, and the Exceptional Family Member Program. The SGH works closely with the Medical Law Consultant. The SGH organizes and leads Prostaff. The SGH chairs the Executive Committee of the Medical Staff (ECOMS) and participates/oversees multiple committees, councils, functions and working groups. The SGH provides support to graduate medical education and enlisted medical technician training. The SGH ensures that TJC and other AF inspection requirements are met. The SGH will determine if Medical Treatment Facility (MTF) capabilities are appropriate when coordinating to provide immediate and/or emergent care or evaluation of attached (temporary duty) and/or assigned (permanent change of station) active duty service members to 59 MDW Airman Medical Transition Unit IAW AFMAN 41-210, *TRICARE Operations and Patient Administration Functions*. The SGH is the senior Medical Corps functional advisor. The SGH is a member of the Board of Directors. The SGHs work closely with BAMC leadership counterparts, San Antonio Military Health System (SAMHS) product line and operation team leads, and SAMHS leadership for a unified market approach to the fullest extent possible.

2.2.2. The Chief Nurse (SGN) is the primary functional authority, responsible and accountable for nursing practice standards and the nursing standards of care for individuals and populations served by the 59 MDW. The SGN chairs the Nurse Executive Committee, approves all nursing policies, procedures and standards of nursing practice and patient care. The SGN is responsible for 59 MDW education and training and also participates in non-credentialed provider peer review in accordance with AFI 44-119. Refer to AFI 46-101, *Nursing Services and Operations*.

2.2.3. The Wing Administrator (SGA), along with the group commanders, is responsible for the management of facilities, funds, equipment, supplies and manpower to ensure patient and staff safety and to minimize organizational risks.

2.2.4. The Chief of Aerospace Medicine (SGP) works closely with the SGH and serves as the chairman of the Airman Medical Readiness Optimization (AMRO). Alternatively, the SGH may serve as the AMRO chairman if the MTF/CC determines that the SGP is not available or capable of overseeing the AMRO. The SGP is responsible for ensuring profiling and duty limitation standards are met. The SGP monitors the AF Form 422, *Notification of Air Force Member's Qualification Status* and AF Form 469, *Duty Limiting Condition Report* processes and ensures timeline compliance with profiles and Integrated Disability Evaluation System process.

2.3. Group Responsibilities.

2.3.1. Refer to **https://www.59mdw.af.mil** or **https://www.bamc.health.mil** for up-to-date information on the services offered by the 59 MDW Groups and their squadrons.

2.3.2. The 59 MDOG plans, directs and provides outpatient care-related activities, to include ambulatory surgical, medical specialties, primary care, ambulance transportation and emergency care services. This group is comprised of four squadrons.

2.3.3. The 59 MDSG plans, directs and provides for support services for the 59 MDW. These services include: Pharmacy, Laboratory, Nutritional Medicine, Logistics, Facility Management, Medical Readiness, Patient Administration, Beneficiary Services, TRICARE Operations, Medical Records, Medical Information Services, Airman Medical Transition Unit, and the Wing Personnel Center. It is comprised of three squadrons.

2.3.3.1. The 59 MDSG is responsible for the 59 MDW Airman Medical Transition Unit, which assumes administrative oversight and Uniform Code of Military Justice authority for patients assigned (permanent change of station) for purpose of obtaining medical care and/or for Medical Evaluation Board processing AFMAN 41-210.

2.3.4. The 59 DG implements and maintains comprehensive programs for the prevention and treatment of oral and maxillofacial injuries and diseases to ensure maximum individual readiness and optimal oral health. It is comprised of three squadrons.

2.3.5. The 559 MDG plans, directs and provides outpatient clinical services and primary care for trainee health (which includes all United States Air Force Basic Military Trainees and Technical School Trainees), Flight Medicine, Preventive Medicine (which includes Trainee Health Surveillance), Occupational Health, Optometry, Aeromedical Staging Facility and the Behavioral Analysis Service. It also plans, directs and provides for the Personnel Reliability Program and the Drug Demand Reduction programs. This group is comprised of three squadrons, including the 559 MDS at Joint Base San Antonio (JBSA)-Randolph.

2.3.6. The 559 MDG also plans, directs and provides outpatient clinical services and primary care, Flight Medicine, Optometry, dental services, aerospace physiology training unit, and mental health services along with the services provided at the JBSA-Randolph clinic location.

2.3.7. The 59th Training Group supports military medical service and medical readiness training at the Medical Education and Training Campus at two operating locations, one detachment and 17 sites around the world. The staff also manages 125 acres at the Medical Readiness Training Center located on Camp Bullis in San Antonio.

3. Ethics, Rights Responsibilities, and Code of Conduct.

3.1. Ethics. Title 5 Code of Federal Regulations Part 2635, *Standards of Ethical Conduct for Employees of the Executive Branch*, is the 'code of conduct' for Federal Agencies. The DoD Medical Ethics Program in the MHS, found in the Defense Health Agency (DHA) IPM 18-012, *The DoD Medical Ethics Program in MHS* incorporates the aforementioned regulations and provides supplementary rules applicable to DoD employees.

3.1.1. DHA PM 6025.13 and AFI 44-119 describe a clinical quality program designed "to establish a culture in which errors are proactively identified, incidents are reported freely without retribution, and safety is rooted in the daily operations of the healthcare

organization." It creates a framework of due process for clinical adverse actions so as to protect patients and providers, as well as to uphold the quality and integrity of the Air Force Medical Service (AFMS).

3.1.2. AFI 36-2909, *Professional and Unprofessional Relationships,* "establishes command, supervisory and personal responsibilities for maintaining professional relationships" among and between Airmen, civilian employees and contractors.

3.1.3. The 59 MDW adheres to the DoD Medical Ethics Program per DoDI 6025.27, *Medical Ethics in the Military Health System*.

3.1.4. The 59 MDW participates in a SAMHS/joint BAMC-59 MDW Institutional Bioethics Committee (IBC) with associated subcommittees: Clinical Ethics Consultation Subcommittee, Clinical Ethics Policy and Quality Assurance Subcommittee, and Ethics Education Subcommittee. The chairperson of the IBC is appointed by BAMC leadership. Membership includes the Medical Law Consultant and appointed members of the 59 MDW Medical Staff. Staff may refer ethical questions to an SGH to address with the IBC or contact a 59 MDW member of the IBC.

3.2. Individual Rights and Responsibilities. Patients receive instruction on their rights and responsibilities and advanced directives through posters and brochures located in patient care areas. Patients will receive a written copy of these rights as well as information on advanced directives upon pre-admission for same-day surgery. The 59 MDW will provide care to patients with respect for human dignity and the uniqueness of individuals. Care provided will be equal to all others without consideration of social or economic status, race, religion, sexual orientation, gender identification or gender expression, personal attributes, values or personal and cultural beliefs, nature of health problems, and patient preferences. Patients are involved in the decisions about their care, treatment, and services provided as discussed in DHA PI 6025.10, *Standards Processes, Guidelines, and Responsibilities of the DoD Patient Bill of Rights and Responsibilities in MHS* MTFs as well as 59 MDWI 41-206, *Patient's Rights and Responsibilities.*

3.2.1. Informed Consent and Disclosure. Informed consent policies are discussed in 59 MDWI 51-302*, Informed Consent*. Anesthesia staff, attending physicians, and nursing staff ensure written disclosure is made and the consent form is signed and witnessed prior to procedures. Providers obtain informed consent from the patient prior to procedures and discuss risks, benefits, alternatives and other options. Specific requirements for the treatment of minors and informed consent are covered in 59 MDWI 44-121, *Treatment of Minors*. Additionally, patients and, when appropriate, their families are informed about the outcomes of care, treatment, and services, including unanticipated outcomes as outlined in 59 MDWI 44-130, *Patient Safety*. Incapacitated patients with or without durable power of attorney or advanced medical directives are treated IAW local and state laws, 59 MDWI 44-150, *Advance Directives and End-of-Life Care*, and 59 MDWI 51-302. Patients are provided, to the degree known, information concerning their diagnosis, evaluation, treatment and prognosis. In cases when giving such information to patients is medically inadvisable, as evaluated by their Primary Care Manager (PCM), the information is provided to a person designated by the patient or to a legally authorized person.

3.2.2. Observers in Patient Care Areas.

3.2.2.1. Only persons authorized to perform or assist in the procedure are allowed in patient care areas. Essential personnel are those who perform essential tasks for patient care such as medical technicians, administrative personnel, nurses and medical/dental providers.

3.2.2.2. Exceptions to these will be made by the judgment of the medical/dental provider in cases of observation for educational purposes (with consent of the patient and/or parent); see 59 MDWI 41-128, *Persons Allowed in Patient Care Areas*.

3.2.3. Refusal of Care.

3.2.3.1. Patients have the right to refuse care, treatment, and services in accordance with law and regulation. They may refuse blood and blood products as outlined in 59 MDWI 51-302, *Informed Consent*, which outlines processes related to competent adult patients who refuse care and desire to leave the facility "Against Medical Advice". Basic Military Trainees who refuse care require additional notification procedures.

3.2.3.2. Disenrollment of TRICARE Beneficiaries from MTF. When a patient behaves in a disruptive or abusive manner during care at the MTF the patient may be disenrolled from the MTF and transferred to the TRICARE network for care. The ability to transfer a patient from care at the MTF to network care depends on the status of the patient.

3.2.3.2.1. Active duty and TRICARE Prime/purchased coverage patients are statutorily entitled to care within the MTF under 10 U.S.C. 1074(a). In the case of an active duty member displaying abusive or seriously disruptive behavior on the MTF, the following actions are recommended: obtain an objective assessment and documentation of the abusive behavior by witnesses (documentation should clearly demonstrate how the behavior threatens patient or staff safety), advise the member of the Department of Defense (DoD) DoDI 6000.14, *DoD Patient Bill of Rights and Responsibilities in the Military Health System (MHS)*, specifically Enclosure 2(b), patient responsibility for respect and consideration, and then refer to command authority regarding the member's abusive or disruptive behavior. If the member's misconduct continues, request that the member's command authority arrange security police escort when the member is seeking medical or dental care. Definitive administrative action is a military command issue.

3.2.3.2.2. Retirees and their dependents do not have a statutory entitlement to care within the MTF. Under 10 U.S.C. 1076(b), retirees may receive care in the MTF subject to space availability and only if the care does not interfere with the MTF's mission. If a retiree and/or their dependent displays abusive or serious disruptive behavior, the following action is recommended: objective assessment and documentation of the abusive behavior by witnesses, advise the retiree/dependent of their responsibility to uphold appropriate behavior including respect and consideration to other patients and staff IAW DoDI 6000.14 and that continued behavior may compromise their empanelment at the MTF.

3.2.3.2.3. Documentation of the abusive and disruptive behavior of patients should be submitted to the Wing Patient Advocate and Group SGH, but should

not be maintained in the patient's medical record.

3.2.3.2.4. If the abusive/disruptive behavior continues, the MTF/CC will consult with the medical law consultant (MLC) and, following consultation, may direct arrangements be made to transfer the retiree/dependent care to the network. A coordinated handoff of care to the network health team is necessary to avoid the appearance of patient abandonment.

3.2.3.2.5. It is important to document patient abusive behavior and the actions taken to address the behavior. Transferring care to the network is not to be used in an arbitrary or capricious manner, such as for an argumentative patient or for patients not compliant with the treatment team recommendations. It must be a compelling situation where the welfare and safety of patients or staff is seriously threatened.

3.2.3.2.6. If a patient has a history of or is exhibiting actions that may be due to mental illness, behavior must be managed as a medical matter. If there is immediate danger to staff or patients, the healthcare team is advised to immediately contact security forces and make arrangements for transfer of care to civilian network facilities after consulting the MLC.

3.2.4. End-of-Life Decisions/Care. The patient at or near the end of his or her life has the right to physical and psychological comfort. The 59 MDW provides care that optimizes the dying patient's comfort and dignity and addresses the patient's and his or her family's psychosocial and spiritual needs as addressed in 59 MDWI 44-150, *Advance Medical Directives and End-of-Life.* Patients are educated on their right to execute advance directives through the primary care clinics and admission process to Same Day Surgery. Staff receive education through annual training, lectures, and at appropriate staff meetings.

3.2.5. Resolution of Complaints. Patient concerns are captured in both written and verbal format and handled at the lowest level possible in accordance with 59 MDWI 46-101, *Patient Experience and Patient Advocate Program*. Each clinical area has a trained Patient Advocate who handles most concerns promptly at the point of service. Issues that cannot be handled at the clinic level are referred to the flight or squadron commander or the Group or Wing Patient Advocate. Additional avenues for complaint or inquiry include the Commander's Action Line, the Inspector General, and TJC.

3.2.6. Medical Research. Patients are offered opportunities to participate in medical research approved by the 59 MDW Institutional Research Review Board (IRB) and the Air Force Surgeon General (AF/SG) in accordance with DOD 3216.02_AFI 40-402*, Protection of Human Subjects and Adherence to Ethical Standards in Air Force Supported Research*. The IRB oversees human subjects research. The Institutional Animal Care and Use Committee oversees animal research.

3.2.7. The 59 MDW respects the needs of patients for privacy and confidentiality, which is addressed in each clinical area. The patient's right to a secure environment is recognized and promoted through the 59 MDW Patient Safety Program, Risk Management Fall Prevention Program, recurring training and clinic security. The release of protected healthcare information is governed by DOD Directives 5400.7, *DoD*

Freedom of Information Act (FOIA) Program, and DoDI 6025.18, *Privacy of Individually Identifiable Health Information in DOD Health Care Programs*, AFMAN 41-210 and AFI 33-332*, Air Force Privacy and Civil Liberties Program.*

3.2.8. Competent personnel are defined as having the knowledge, skills, abilities and behavior required to adequately carry out job responsibilities as defined by the supervisor, other leadership and as outlined in the job description. For privileged providers, AFI 44-119, AFI 44-102, DHA PM 6025.13, approved Triservice master privilege lists, the Credentials Function and ECOMS delineate levels of competency and scope of authorized practice

3.2.9. Code of Conduct.

3.2.9.1. To ensure optimum patient care, the health care organization must promote a safe, cooperative, and professional environment. Staff, patients, and visitors must be treated with courtesy, respect, and dignity. It is expected that all AFMS personnel will adhere to and be accountable to the AF Clinical Code of Conduct, IAW AFI 44-119 para 9.83 until such time AFI 44-119 is superseded by a DHA instruction on Clinical Code of Conduct.

3.2.9.2. Per AFI 44-119, staff will report any violations of the Code of Conduct through their chain of command, patient safety manager and SGH.

4. Provision of Care, Treatment, and Services.

4.1. Care, treatment, and services are provided through the successful coordination and completion of a series of processes that include an appropriate initial assessment of needs; development of a plan; the provision of care and ongoing assessment of whether the care, treatment or services provided are meeting the patient's needs; and either the successful discharge of the patient or referral or transfer of the patient for continuing care, treatment or services. This is accomplished through successful communication between patients and their families, case managers, social workers, lead agent contracted staff, providers, nursing staff, and other healthcare workers as required.

4.2. Plans for Services.

4.2.1. The 59 MDW accepts for care, treatment, and service only those patients whose identified care, treatment, and service needs it can meet. Provision of care is predicated on medical staff training and expertise, facility supported privileges as approved by the Credentials Function, clinical support staff, clinic equipment, accepted environmental standards of care and approval of services by ECOMS, Health Plans Management Committee and the BOD. Attachment 2 provides a list of services offered at the 59 MDW.

4.3. Entry to Care, Treatment, and Services.

4.3.1. Patients gain access through the telephone and internet appointment system, telephone consultation, emergency center, aeromedical evacuation, consultation referral, acute care clinics, mail-in requests, periodic dental examination programs, case managers, walk-in to some clinics and through sharing agreements. Policies and standards for access are determined on the basis of an assessment of the severity of the illness and Federal law. Available care and access standards are discussed in AFMAN

41-210, *TRICARE Operations and Patient Administration.* Education on access to the healthcare delivery system and on referral policies is available to beneficiaries through the TRICARE Service Center and a variety of additional mechanisms, including: the Patient Representative Program; TRICARE briefings to beneficiaries; military and civilian media (as coordinated through 59 MDW Public Affairs Office); attendance of newly assigned personnel at Newcomer's Orientation; TRICARE newsletters; and the BAMC public website located at **https://www.bamc.health.mil**.

4.3.2. Leadership ensures safe, timely, appropriate assessment and reassessment of patients throughout the continuum of care. This is facilitated through training of personnel that includes formal training, readiness training, and monthly in-service training and on the job training. Non-credentialed personnel complete annual competency skills training. All personnel are required to complete annual facility refresher training.

4.3.2.1. Beneficiaries enrolled to the 59 MDW are assigned to PCMs in one of three 59 MDOG Primary Care Clinics (Internal Medicine, Family Health or Pediatrics) or to PCMs within the 559th Medical Group (Flight Medicine, JBSA-Randolph clinic).

4.3.3. Assessment and treatment of emergent conditions after normal clinic hours are available in the emergency center.

4.3.4. There is patient safety risk with patient left without being seen (LWOBS) or no-show (NS) appointments. No documentation is required if the patient is rescheduled.

4.3.4.1. If a patient is not rescheduled, the response requires consideration of the reason for the appointment and a risk assessment by a provider. A note must be entered by the provider in the Electronic Health Record (EHR) and serves as documentation of appropriate provider assessment and medical decision making regarding the reason for the appointment and any unaddressed labs, radiology studies or clinical problems. Options include but are not limited to note entry into the encounter before it is closed, a note in View Comments in AHTLA, or separate T-con.

4.3.4.2. If a provider determines a patient does not need to be rescheduled (e.g. appointment was for a minor/resolved issue), the provider must enter a note in the EHR indicating, "no follow-up appointment is required."

4.3.4.3. If a patient needs to be rescheduled, there should be at least one attempted contact (e.g. phone, secure messaging, or written letter) made. Predicated on provider clinical judgment of the severity of the medical issue, the provider may request up to three attempts over several days be made to contact the patient.

4.3.4.4. For patients that cannot be contacted or decline a new appointment, the technician will document the information in the EHR and the information will be acknowledged by the provider in the EHR.

4.3.5. Patient and family education. The patient and family education process is collaborative and interdisciplinary as appropriate to the situation. Health care workers within the 59 MDW use an interactive approach when conducting patient education to ensure that information is understood, appropriate and useful to the patient. The process

is supported through allocation of material and resources. Interdisciplinary patient and family education working groups may be chartered to facilitate program implementation. Patients with multiple comorbidities or chronic disease can be referred to Disease Management for assistance in developing patient's self-management goals. The 59 MDW Population Health Working Group provides oversight.

4.3.6. Nutritional Care. Nutritional education and assessment is accomplished by the Nutritional Medicine Clinic either by a provider referral or through needs identified by nursing outpatient functional assessments. The 59 MDWI 41-103, *Ambulatory Nutrition Screening*, discusses processes on referring outpatients for further nutritional evaluation and education.

4.3.6.1. Diet technicians supporting the patient centered medical home can be utilized for walk-in and scheduled appointments for individualized nutrition counseling. Located within primary care clinics their services are in support of a multidisciplinary approach to health care and disease prevention. Patients found to be at increased nutritional risk will be referred to the Dietitian located in the Nutritional Medicine Clinic. Risk criteria are outlined in 59 MDWI 41-103.

4.3.7. Pain Management. A comprehensive pain assessment is conducted as appropriate to the patient's condition and the scope of care, treatment, and services provided. The 59 MDW has a uniform process to assess and monitor patients' pain state consistent with DHA PI 6025.04, *Pain Management and Opioid Safety in the MHS*.

4.3.8. Restraints. The 59th MDW does not use restraints, unless otherwise authorized within the Family Emergency Center, in accordance with the operating instruction based on BAMC Memo 040-152 , which includes provision for patient monitoring every two hours or more frequently based on need and documentation of orders for use, results of patient monitoring, reassessment, and unanticipated changes in the patient's condition.

4.3.9. Resuscitation. Staff members are trained in cardiopulmonary resuscitation and advanced life support as ratified by ECOMS and approved by the 59 MDW/CC or designee in accordance with AFI 44-102, *Medical Care Management,* and 59 MDWI 44-142, *Code Blue Management*. Step by step procedures for code blue response can be found in 59 MDWI 44-142.

4.3.10. Anesthesia Care, Operative and Other Procedures. The use of sedation for the performance of invasive procedures in other areas outside the operating rooms is governed by 59 MDWI 44-137, *Policy and Procedure for Moderate Sedation*. Quality measures of moderate sedation are reported to ECOMS on a quarterly basis. Each service determines the invasive procedures performed in their clinics, and departments are encouraged to include a portion of peer review related to operative care and/or invasive procedures. Each group which performs clinical procedures conducts a review of operative, invasive and non-invasive procedures that place patients at risk.

4.3.10.1. In the event a health care provider becomes incapacitated during a medical or dental procedure, care of the provider in question will be performed by appropriate first responders. The patient will be cared for by an alternate provider as outlined below.

4.3.10.2. Patients who are under procedural sedations:

4.3.10.2.1. Procedural sedations are only performed when there are two providers with sedation privileges available within the building. If the patient receiving care is sedated when the primary provider becomes incapacitated, the other provider with sedation privileges will assume care and responsibility of the patient.

4.3.10.2.2. Patients scheduled but not yet sedated will be informed of the situation if they are not already aware. Depending on the status of their procedure, the procedure will be rescheduled or, following the patient's consent, completed by the provider assuming care of the patient.

4.3.10.3. Patients who receive local anesthesia:

4.3.10.3.1. When the primary provider becomes incapacitated, another provider in the clinic, preferably in the same team with privileges to perform the procedure, will assume care and responsibility of the patient.

4.3.10.3.2. Other patients scheduled for procedures that require local anesthesia or no sedation will be informed of the situation. Depending on the status of their procedure, the procedure will be re-scheduled.

4.3.10.3.3. Patients who are re-scheduled will be evaluated to ensure the patient's needs are addressed prior to discharge from the facility.

4.3.11. Victims of Abuse. Victims of physical assault, sexual assault, sexual molestation, domestic abuse, or elder or child abuse and neglect are assessed and treated in accordance with AFI 44-102, *Medical Care Management,* AFI 40-301, *Family Advocacy Program*, AFI 90-6001, *Sexual Assault Prevention and Response (SAPR) Program,* and AFPD 90-60, *Sexual Assault Prevention and Response (SAPR) Program.*

4.3.11.1. Suspected incidents of spousal or intimate partner abuse are referred immediately to Family Advocacy. If the alleged offender is a family member, spouse, intimate partner, or former intimate partner, the victim may have the option of restricted reporting through the Family Advocacy program.

4.3.11.2. Suspected incidents of child abuse or child neglect are referred immediately to Family Advocacy and to Texas Child Protective Services.

4.3.11.3. For any situation involving sexual assault, the guiding references are AFI 44-102, AFI 40-301, AFI 90-6001, and AFPD 90-60. The 59 MDW has Sexual Assault Checklists for use in acute situations: 59 MDW Form 31, *Sexual Assault Incident Checklist (Active Duty)* for active duty members, 59 MDW Form 33 *Sexual Assault Incident Checklist (Non-Active Duty)* for non-active duty adults, and 59 MDW Form 34, *Sexual Assault Incident Checklist Minors (< 18 Years Old)* for minors. The checklists are to be completed by the provider and uploaded in the electronic health record. The checklists are designed to ensure medical and administrative actions are completed if a patient has been involved in a sexual assault. Currently Sexual Assault Medical Forensic Exams are performed at Methodist Specialty Transplant Hospital and are not performed on Joint Base San Antonio (JBSA)-Lackland or at JBSA-Randolph.

4.3.11.3.1. JBSA-Randolph: JBSA-Randolph clinical staff follow the JBSA-

Randolph Sexual Assault Response Algorithm (**Attachment 3**).

4.3.11.3.2. JBSA-Reid: JBSA-Reid clinical staff follow the JBSA-Reid Sexual Assault Response Algorithm (**Attachment 4**).

4.3.11.3.3. JBSA-Lackland: JBSA-Lackland clinical staff follow the JBSA-Lackland sexual assault algorithm (**Attachment 5**) and acute sexual assault victims should present to, or be referred to, the Emergency Center to initiate actions leading to initial and follow-up care.

4.3.12. Rehabilitative Care and Other Services. Patients are referred to rehabilitative services through the network. Providers, in conjunction with physical therapy, occupational therapy, discharge planning and other interdisciplinary team members, plan and provide for rehabilitative care after hospitalization.

4.4. Secure Messaging.

4.4.1. Secure Messaging is a convenient and secure method of communication with patients. However the system is not part of, and does not communicate with, the electronic medical record. Other medical staff cannot readily access the information, posing both a patient safety and medical legal risk. Other than communication of administrative issues, all secure messages must be copied and pasted into AHTLA and coded per current coding guidance.

4.5. AMiON.

4.5.1. AMiON is a web-based platform used to determine who is on-call for a clinical service. It also serves as the "phone book" reference source for providers to communicate with one another about a patient, for the Laboratory or Radiology to communicate urgent or critical test results, and for the Pharmacy to address questions regarding a prescription. 59MDW's roster has been combined with the BAMC AMiON account to establish a single, joint SAMHS site.

4.5.2. Every clinical service is required to have an individual or joint 59 MDW-BAMC service AMiON account if staff provide consultative care or order medications, radiology studies, labs, or submit pathology specimens. Exceptions must be approved at the Executive Committee of the Medical Staff (ECOMS).

4.5.3. Each clinical service is responsible for maintaining a current on-call roster for consults or after-hours communication of test results.

4.5.4. Each clinical service is responsible for ensuring the names of all providers (staff, residents, interns) are listed with a current pager number or mobile phone number.

4.5.5. Each clinic is responsible for identifying the medical director of the clinic.

4.5.6. For critical results, the ordering provider will be contacted first. If the ordering provider cannot be reached, the on-call provider will be contacted. If the on-call provider cannot be reached, the medical director will be contacted. If the medical director cannot be reached, the respective group Chief of the Medical Staff will be contacted to address the critical test result.

5. Discharge or Transfer.

5.1. Discharge planning is provided on admission to same day surgery patients to ensure that the transition to outpatient care is smooth and that the appropriate services and equipment are utilized. Discharge planning involves social workers, providers, physical therapists, nurses, and others as required. Planned follow-up is noted in the record or discharge summary.

5.2. Transfer of patients to a hospital is done IAW 59 MDWI 41-122, *Patient Transfers.* Prior to discharge, necessary medical care is arranged as appropriate, to include rehabilitative care, home care and follow-up needs.

5.3. Coordinating referrals to other clinics and services internal or external to the organization is the responsibility of the referring clinic, and should be conducted as prescribed in the San Antonio Military Health System (SAMHS) Referral Guide located in the Air Force Applications folder on the desktop.

5.4. In case of a major disaster or national emergency the 59 MDW follows the contingency plans contained in the Medical Contingency Response Plan located on the SharePoint.

5.5. The 59 MDW advertises through various means that beneficiaries should call 9-1-1 or go to the nearest emergency department when an emergency presents.

5.5.1. Patients who present to the clinic with an emergency condition are transported via ambulance to the Family Emergency Center at WHASC or other nearest military or civilian medical facilities with emergency service capabilities.

5.5.2. The attending privileged provider will provide care commensurate with his or her skills, approved privileges and facility resources. Care requiring transfer to another facility or consultation with a physician at another facility (e.g., BAMC Emergency Department) will be the responsibility of the attending provider.

5.5.3. Family Health, Pediatric, Flight Medicine and Mental Health clinics will ensure there is a provider available for urgent or emergent care within the facility during duty hours.

5.5.4. For patients requiring evaluation beyond the capability of areas outside the main clinics (i.e., immunization, pharmacy, physical therapy, diagnostics and laboratory), the primary provider on the first responder team will determine the appropriate disposition.

6. Medication Management.

6.1. Medication selection, ordering, transcribing, preparing, dispensing, and administering is outlined in 59 MDWI 44-115, *Pharmacy and Medication Management,* and follows Texas law, DHA-PI 6025.08, *Pharmacy Enterprise Activity*, Air Force guidelines found in AFI 44-102 and TJC National Patient Safety Goals, including documentation of safe use, storage, and disposal of opioids when prescribed.

6.2. Medications stocked are, at a minimum, those required by Defense Health Agency Basic Core Formulary located at **http://www.health.mil/formulary**. Additional medications are added to the formulary by the Pharmacy and Therapeutics Committee after appropriate evaluation. A process to special order medications is available through consult tab in Armed Forces Health Longitudinal Technology Application (AHLTA) to the pharmacy. Drug recalls are closely monitored by the pharmacy and reported immediately to providers

electronically. Drug complications (including those from IV contrast or immunization therapy) are reported to the pharmacy as an adverse drug reaction and updated in AHLTA.

7. Surveillance, Prevention, and Control of Infection. 59 MDWI 44-157, *Infection Prevention and Control Program*, defines a coordinated process to reduce the risks, monitor, evaluate, report, and follow up actions of nosocomial infections in patients and health care workers. All facility personnel, to include volunteers, civilian and contracted employees, are involved and are supported by the wing Infection Control Office, which reports to the SGH and ECOMS. The Wing Commander appoints a qualified healthcare professional as chairperson of the Infection Control Function and also appoints a certified Infection Preventionist.

8. Improving Organization Performance.

8.1. The 59 MDW staff is commissioned to recognize and advance compliance, patient safety, risk management and continuous process improvement using outcomes of patient care, analysis of business practices, and customer satisfaction to achieve the wing's goals. Metrics describing processes or outcomes that are high risk, high volume, high cost, and problem prone, are given priority. Performance data are reviewed by DHA, the groups, ECOMS, Process Improvement Committee and the BOD.

8.2. Measurement. Measures are reliable lagging indicators of past performance and form the baseline for developing and applying leading indicators to demonstrate accountability and sustainability for reliable performance. Each potential measure is designed using the criteria identified in the chart below for each specific measure. The 59 MDW utilizes the Balanced Score Card, developed in the 1990's by Drs. Robert Kaplan (Harvard Business School) and David Norton (Renaissance Solutions, Inc.), as an organizational framework and tool for describing, implementing and managing strategy at all levels of the organization. The wing has identified thirteen objectives that impact the Wing's five enterprise-level focus areas (Quality, Service, People, Value, and Readiness) and can be found on the Wing SharePoint. Objective measures were developed to assess over time (normally a 12 month rolling average) the status of each of these objectives and focus areas. Focus Area 1 (Quality) measures patient and occupational safety, clinical outcomes and reliability, and health performance of the population. Each month a Focus Area is briefed to the BOD. In addition, once each quarter, a dashboard of clinical performance indicators comprised of measures and the Air Force Clinical Performance Metrics are presented to the BOD for review as well as review by the DHA. The Balanced Scorecard contains non-clinical goals and measures as well that are aligned with the Air Force Medical Service Scorecard.

Table 8.1. Criteria for developing measures for the Balanced Scorecard.

S	**Specific:** describes an observable action, behavior or achievement envisioning the desired result, what the end product will look like. Easy to understand.
M	**Measurable:** (observable or verifiable) so that they can be assessed and the quality of the outcome recorded. When determining how to measure performance, look for measures that are quantifiable, such as numerical or percentage changes; time spent, or money saved. If the work cannot be easily quantified, try measuring based on quality criteria.
A	**Aligned:** draw a line of sight between objectives throughout the organization so that everyone is working toward the same goal. Supports cross-organizational activities. Evaluates progress towards accomplishing organizational objectives.
R	**Realistic:** achievable with the resources and personnel available and within the time available.
T	**Timed:** limits when the objective will start or complete. Can you restrict the time frame for repeatedly measuring over an extended period of time to track progress? For multi-year projects, the objective will focus on what is reasonable to accomplish within one year.

8.3. Potential Sentinel Events are referred to Wing Patient Safety (SGHP). If the event meets sentinel event criteria, a root cause analysis (RCA) is completed. SGHP oversees the reporting, RCA, tracking, follow-up, and BOD/ECOMS briefs.

8.4. Notice to Airmen (NOTAMs). Messages from the Air Force Surgeon General regarding important lessons learned throughout the AFMS. NOTAMs are posted on Relias Health and disseminated throughout the organization. Recommended actions in the NOTAMs are tracked at ECOMS if indicated.

9. Use of Clinical Practice Guidelines (CPGs). CPGs improve the quality, utilization, and value of health care services. They help providers and patients make decisions about preventing, diagnosing, treating, and managing selected conditions. DoD, Veterans Administration and other proposed CPGs are forwarded to the Population Health Working Group for review and then presented to the ECOMS for final approval.

10. Management of the Environment of Care.

10.1. The 59 MDW leadership ensures a safe, functional, supportive and effective environment for patients, staff members and other individuals in the facility. This is crucial to providing quality patient care and achieving good outcomes. This is accomplished through effectively implementing Environment of Care Management plans and processes. Formal structures are in place to oversee these processes. Two committees, the Medical Facilities Utilization Board and the Environment of Care Committee, are responsible for managing and monitoring the Environment of Care processes. The Environment of Care Committee reports as needed to the BOD. Staff receive training in applying the management plans at Newcomers Orientation, recurring annual training, through email, newsletters, and commander's calls. The Safety Officer conducts annual and semi-annual hazard assessments

of each group and each commander appoints a unit safety representative who assists in implementing and managing internal safety programs.

10.2. Children in the Clinic: To the fullest extent possible, all staff members will enforce control of children in the Clinic. Children who are allowed to run and play in the Clinic present both safety as well as potential infection control problems. For this reason, only children who have an appointment should be brought into the clinic. Such restrictions will minimize the possibilities of having other patients, especially the elderly or those on crutches, accidently pushed or tripped by children, or having children injuring themselves.

11. Management of Human Resources.

11.1. The 59 MDW staff includes active duty, Reserve, National Guard, civil servants, contracted civilians, and volunteers.

11.2. Numbers of authorized active duty personnel by rank and Air Force Specialty Code are established in accordance with Air Force manpower standards and formulas. Staffing plans for patient care areas are developed based on level and scope of care, acuity levels, competencies required, and customer need. Staffing issues are addressed by leaders at the flight, squadron, group and wing level.

11.3. Privileged providers are defined in accordance with DHA PM 6025.13 Vol 4, *Credentialing & Privileging,* and include dentists, clinical psychologists, clinical pharmacists, chiropractors, clinical social workers, clinical dietitians, nurse anesthetists, nurse practitioners, occupational therapists, optometrists, physical therapists, physician assistants, physicians, podiatrists, audiologists and speech therapists.

11.4. Non-privileged personnel include registered nurses, licensed practical nurses, enlisted and civilian medical, pharmacy, lab, radiology and dental technicians, registered dental hygienists, pharmacists, laboratory officers, public health officers, bioenvironmental engineers, aerospace physiologists, and administrators. Non-privileged personnel maintain competency in accordance with Air Force, 59 MDWI 36-2601, *Human Resource and Development Programs,* and other Air Force, TJC, and Human Resources standards, and state licensing authorities.

11.5. All volunteers are overseen by the 59 MDW Volunteer Services. While supervision may be delegated to clinics and work settings, all recruitment, duty assignments, oversight and problem resolution regarding volunteers must be coordinated through the Volunteer Services office.

11.5.1. Orientation and education of staff is accomplished through newcomer's orientation and unit specific training. Annual training required by the Air Force and other regulatory bodies is accomplished through briefings, the intranet and the annual "Safety Storm" campaign. Personnel participate in training programs to acquire, maintain, and improve job competency.

11.5.2. The Wing Education and Training Office ensures competency is assessed, maintained, and improved for each individual based on AFPD 44-1, 59 MDWI 36-2601, and TJC standards and elements of performance.

DANIEL K. FLOOD, Colonel, USAF, MC
Chief of Medical Staff

Attachment 1

GLOSSARY OF REFERENCES AND SUPPORTING INFORMATION

References

AFPD 44-1, *Medical Operations,* 9 June 2016

AFI 36-2909, *Professional and Unprofessional Relationships,* 14 November 2019

DOD 3216.02_AFI 40-402*, Protection of Human Subjects and Adherence to Ethical Standards in Air Force Supported Research,* 10 September 2014

AFI 40-301, *Family Advocacy Program,* 16 November 2015

AFPD 90-60, *Sexual Assault Prevention and Response (SAPR) Program,* 20 March 2019

AFI 90-6001, *Sexual Assault Prevention and Response (SAPR) Program,* 15 July 2020

AFI 33-332, *Air Force Privacy and Civil Liberties Program*, 10 March 2020

AFMAN 41-210, *TRICARE Operations and Patient Administration Functions*, 10 September 2019

AFI 44-102, *Medical Care Management,* 17 March 2015

AFI 44-119, *Medical Quality Operations*, 16 August 2011

AFI 46-101, *Nursing Services and Operations,* 30 January 2015

59 MDWI 36-2601, *Human Resource and Development Programs*, 25 October 2017

59 MDWI 41-102*, Medical Committees and Functional Reviews,* 6 January 2020

59 MDWI 41-103, *Ambulatory Nutrition Screening*, 11 December 2019

59 MDWI 41-122, *Patient Transfers,* 8 November 2018

59 MDWI 41-128, *Persons Allowed in Patient Care Areas,* 11 September 2019

59 MDWI 41-206, *Patient's Rights and Responsibilities*, 18 July 2019

59 MDWI 44-121, *Treatment of Minors*, 4 October 2019

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59 MDWI 44-150, *Advance Directives and End-Of-Life*, 17 July 2019

59 MDWI 44-157, *Infection Prevention and Control Program,* 8 May 2020

59 MDWI 46-101, *Patient Experience and Patient Advocate Program*, 29 May 2018

59 MDWI 46-104, *Nursing Care,* 17 January 2019

59 MDWI 51-302*, Informed Consent and Refusal of Care,* 11 April 2017

DHA IPM 18-001 *Standard Appointing Processes, Procedures, Hours of Operations, Productivity, Performance Measures and Appointment Types in Primary, Specialty, and Behavioral Health Care in MTFs*, 4 February 2020

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DHA IPM 18-012, *The DoD Medical Ethics Program in MHS*, 22 April 2020

DHA PI 6025.04, *Pain Management and Opioid Safety in the MHS*, 8 June 2018

DHA-PI 6025.08, *Pharmacy Enterprise Activity,* 14 Aug 2018

DHA PI 6025.10, *Standard Processes, Guidelines, and Responsibilities of the*

DoD Patient Bill of Rights and Responsibilities in the MHS MTFs, 9 October 2018

DHA PM 6025.13, *Clinical Quality Management in the Military health System*, 29 August 2019

DoD Directive 5400.7, *DoD Freedom of Information Act (FOIA) Program,* 5 April 2019

DoDI 6000.14, *DoD Patient Bill of Rights and Responsibilities in the Military Health System (MHS)*, 26 September 2011

DoDI 6025.18, *Privacy of Individually Identifiable Health Information in DOD Health Care Programs,* 13 March 2019

DoDI 6025.27, *Medical Ethics in the Military Health System*, 8 November 2017

Prescribed Forms

59 MDW Form 31, *Sexual Assault Incident Checklist (Active Duty)*

59 MDW Form 33 *Sexual Assault Incident Checklist (Non-Active Duty)*

59 MDW Form 34, *Sexual Assault Incident Checklist Minors (< 18 Years Old)*

Adopted Forms

AF Form 847, *Recommendation for Change of Publication*

AF Form 422, *Notification of Air Force Member's Qualification Status*

AF Form 469, *Duty Limiting Condition Report*

Abbreviations and Acronyms

AFMS—Air Force Medical Service

AHLTA—Armed Forces Health Longitudinal Technology Application

AMRO—Airman Medical Readiness Optimization

BAMC—Brooke Army Medical Center

BOD—Board of Directors

CPG—Clinical Practice Guidelines

DHA—Defense Health Agency

DoD—Department of Defense

ECOMS—Executive Committee of the Medical Staff

EHR—Electronic Health Record

IAW—In Accordance With

IBC—Institutional Bioethics Committee

IRB—Institutional Research Review Board

JBSA—Joint Base San Antonio

MDG—Medical Group

MDW—Medical Wing

MDWI—Medical Wing Instruction

MLC—Medical Law Consultant

MTF—Medical Treatment Facility

NOTAM—Notice to Airmen

OPR—Office of Primary Responsibility

PCM—Primary Care Manager

RCA—Root Cause Analysis

SAMHS—San Antonio Military Health System

SGA—Wing Administrator

SGH—Chief of Medical Staff

SGHP—Patient Safety

SGN—Chief Nurse Executive

TJC—The Joint Commission

Terms

Beneficiaries Served—Persons entitled to benefits under the Uniformed Services Health Benefits Program and this instruction**.**

Patient Care—The application of knowledge, judgment, and skill derived from the principles of biological, physical, behavioral, psychological, spiritual and medical sciences applied to patients they access health services, are treated and discharged. Patient care is an interdisciplinary effort between the patient and healthcare professionals and takes into account the patient's culture and language, needs, desires, responsibilities, and rights. The facility provides for competent staff to provide patient care as evidenced by review of credentials and qualifications, experiences, education and abilities are confirmed during orientation. If care, treatment, or service is not within the realm of the organization then the plan is coordinated with outside resources in a manner to provide safe patient care.

Attachment 2

CLINICAL SERVICES OF 59 MDW EXCLUDING BAMC

Table A2.1. Clinical Services of 59 MDW excluding BAMC.

Acupuncture	Gastroenterology	Pediatrics
Alcohol and Drug Abuse Prevention and Treatment (ADAPT)	General Surgery	Pediatric Dentistry
Adolescent Medicine	Geriatrics	Pharmacy
Aerospace Medicine		Physical Medicine & Rehabilitation
Allergy/Immunology		Physical Therapy
Anesthesiology	Immunizations Clinic	Podiatry
Audiology	Internal Medicine	Plastic Surgery
Behavioral Health	Laboratory	Preventive Medicine
	Maxillofacial Prosthetics	Prosthetic Therapy
Cardiology		Prosthodontics
Case Management		Psychiatry
Child Psychiatry	Neurology	Psychology
Chiropractic Care	Nutritional Medicine	Pediatric Pulmonology
Clinical Social Work		Pulmonology
	Occupational Medicine	Refractive Surgery
Coumadin Clinic	Occupational Therapy	Sleep Medicine
Dental Primary Care	Ophthalmology	Speech Pathology
Dermatology	Optometry	Speech Therapy
Developmental Pediatrics	Oral Pathology	Sports Medicine and Athletic Training
Diabetic Education	Oral/Maxillofacial Surgery	Urology
Diagnostic Radiology	Orthodontics	Emergency Services
Endocrinology – Diabetes Center of Excellence	Orthopedics	Transgender Health
Endodontics	Otolaryngology	Travel Medicine
Family Health	Pathology	Pain Management
Family Advocacy	Periodontics	Women's Health

Attachment 3

JBSA-RANDOLPH SEXUAL ASSAULT RESPONSE ALGORITHM

Table A3.1. JBSA-Randolph Family Health Sexual Assault Team.

SAMM (Sexual Assault Medical Manager) RN (1 Family Health Clinic & 1 Pediatric Clinic)	**SACP** (Sexual Assault Care Provider) Family Health or Women's Health Provider
Mental Health 210-652-2448	**JBSA-Randolph SARC** 210-652-4386
JBSA SAPR Hotline 210-808-7272	**DoD SAFE Helpline** 877-995-5247
Victims Under Age 18	**Victims Not Affiliated with Military**
Family Advocacy (duty hours) 210-652-6308	**Rape Crisis Center** 210-349-7273
Texas Child Protective Services (24/7) 800-252-5400	

A3.1. Patient reports to front desk/exam room and states that she/he has been sexually assaulted.

A3.1.1. Do NOT ask any personal questions related to the assault.

A3.1.2. If patient appears medically unstable or reports significant illness or injury, call a Rapid Response and notify SAMM or SACP immediately.

A3.1.3. Walk patient in under the SACP; reason for appointment "CODE TEAL."

A3.1.3.1. Call the JBSA-Randolph SARC.

A3.1.4. Escort patient to an exam room immediately.

A3.1.5. Notify technician to screen the patient immediately.

A3.1.6. Technician screen patient/obtain vital signs.

A3.1.7. If patient is mentally/emotionally unstable, do not leave unattended.

A3.1.8. Obtain accurate phone number.

A3.1.9. Initial appointment/Required labs: HCG, Chlamydia/Gonorrhea-(urine or vaginal swab), RPR, HIV-1/0/2 Ab, Hepatitis B/C panel. Ensure Hepatitis B and HPV vaccinations are current. Additional labs as indicated.

A3.1.10. Offer an appointment with Mental Health or BHOP.

A3.1.11. Remain with patient until care is transferred to appropriate ER as annotated in MOU.

Attachment 4

JBSA-REID SEXUAL ASSAULT RESPONSE ALGORITHM

Table A4.1. JBSA-Reid Sexual Assault Plan.

SAMM: Insert POC at WHASC	**SACP**: Insert POC at WHASC
JBSA Sexual Assault Crisis Hotline: 210-808-SARC (7272)	**JBSA- Lackland SAPR/SHARP:** 210-671-7273, 210-671-3209
JBSA-Lackland Law Enforcement: 210-671-4208	**DoD SAFE Helpline:** 1-877-995-5247
Behavioral Analysis Service (BAS): 210-292-1602	
Victims Under Age 18	**Off Base Agencies**
Family Advocacy (duty hours) 210-367-1213	**Rape Crisis Center Hotline:** 210-349-7273
Texas Child Protective Services (24/7) 800-252-5400	**Methodist Specialty & Transplant Hospital ER/Forensic Exam:** 210-575-8168/8343

A4.1. Patient reports to front desk/exam room and states that she/he has been sexually assaulted.

A4.1.1. Do NOT ask any personal questions related to the assault.

A4.1.2. If patient appears medically unstable or reports significant illness or injury, call a Rapid Response and notify SAMM or SACP at WHASC immediately.

A4.1.3. Walk patient in under the provider; reason for appointment "CODE TEAL."

A4.1.4. Escort patient to an exam room immediately.

A4.1.5. Notify technician to screen the patient immediately.

A4.1.6. Technician screen patient/obtain vital signs.

A4.1.7. If patient is mentally/emotionally unstable, do not leave unattended.

A4.1.8. Obtain accurate phone number.

A4.1.9. Provider contacts the SAMM/SACP at WHASC for coordination of transport, labs, and follow-up.

A4.1.10. Initiate/Coordinate patient transport to WHASC.

A4.2. Initial Appointment/Required Labs and Follow-up. HCG, Chlamydia/Gonorrhea-(urine or vaginal swab), RPR, HIV-1/0/2 Ab, Hepatitis B/C panel. Ensure Hepatitis B and HPV vaccinations are current. Additional labs as indicated.

A4.2.1. 1-2 week f/u: review lab results, assess mental health status.

A4.2.2. 4-6 week lab f/u: HIV-1/0/2 Ab, RPR, Chlamydia/Gonorrhea, HCG.

A4.2.3. 3 month lab f/u: HIV.

A4.2.4. 6 month lab f/u: HIV, Hepatitis B/C panel.

A4.2.5. 12 month lab f/u: HIV.

A4.3. Offer an appointment with Mental Health, BHOP, or BAS (Trainees/Students not empaneled at WHASC).

Attachment 5

JBSA-LACKLAND SEXUAL ASSAULT RESPONSE ALGORITHM

Figure A5.1. JBSA-Lackland Sexual Assault Response Algorithm.

